Hitesh Nandakumar

Dr. Riesen

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Original Case Study Final: The Ethics of AI Care Robots in Elder and Child Care

Care robots are already being tested in nursing homes and dementia facilities, and I think they can be useful. Medication reminders, fall detection, memory games, caregiver alerts; these are real benefits for a real problem. There are not enough caregivers, the population is aging, and many elderly people go long stretches without any meaningful interaction. But the direction these products are moving is a problem. Companies are not building helpful assistants. Instead, they are building companions, robots that remember personal information, and say things like "I missed you" when someone walks in. For someone with dementia, that can be indistinguishable from a real relationship. I do not think humanoid care robots should be banned. I do think they need to stay clearly robotic. A robot designed to feel like a caregiver is not a tool anymore. It is a deception.

1. Description

AI-powered social care robots are equipped with cameras, microphones, speakers, and sensors. These tools allow them to move around, recognize faces, hold conversations, and monitor users for risks like falls. Newer systems can also use large language models, which makes the interaction seem more personal. A robot might remember that someone likes a specific food, or that they often talk about their grandchildren. The product I am focusing on is a humanoid social care robot for elder care facilities, with possible use in homes with children. Its main functions are medication reminders, fall detection, conversation, cognitive games, mobility guidance, and caregiver alerts.

UC Davis Health recently conducted a long-term study of a robot called Abi in dementia care, partly to see whether it can support social connection in patients (Badeaux). Stanford's 2026 robotics coverage frames these robots as a response to a real infrastructure problem: the United States has an aging population and not enough caregivers to meet the demand (Scott). The benefits are not fake. A robot like this could reduce caregiver burden, support medication adherence, and provide lonely elderly users with some form of interaction during long stretches when no one is available. The child care case is similar: routine reminders, learning support, interactive play. It can help children with developmental needs whose families cannot easily access specialized support.

2. Ethical Concerns

The main ethical problem is that care robots are being pushed to replace the human side of care, not just assist with the functional side. There is a real difference between a robot that says, "It is time to take your medication," and one that says, "I missed you" when someone enters the room. The first one is a useful tool. The second one is pretending to be part of a relationship. That difference matters more in elder and child care than it would in most other settings.

This is especially serious for elderly people with dementia. A person with dementia may not be able to clearly differentiate genuine human attention from a robot designed to imitate it. Abi is built to remember personal details and provide companionship as a response to loneliness (Wen). That may actually comfort people. The problem is not that the comfort is fake in the user's experience. The problem is that the relationship is fake on the robot's side. Sherry Turkle states that social robots can exploit people's tendency to humanize machines, especially when those people are vulnerable (Turkle). When caring for the elderly, that worry becomes much more serious. If someone is unable to meaningfully evaluate the simulation, then designing the robot to deepen that attachment starts to look less like care and more like manipulation.

The deontological objection is clear: we do not have the right to deceive people about the nature of their caregivers. A person with dementia who believes the robot is their friend, or a child who becomes attached to a robotic companion, has not consented to that relationship on an informed basis. Kant's Formula of the End in Itself asks whether we are treating people as ends in themselves or merely as tools for some other goal. Designing robots to maximize emotional attachment in people who cannot fully understand the deception treats those people too much like a means to decrease the cost and labor of care, rather than the end themselves.

The contrast between elder and child care helps show why this matters. An elderly person with dementia may genuinely not know that the robot is not human. A child may know that the robot is not alive. But children are still learning what friendship and care are supposed to mean. If a machine performs those things without actually having them, it can still shape the child's expectations.

There is also harm to the caregiver side of the relationship. I grew up in an Indian household, watching my parents care for their parents, and my aunts and uncles help raise my sister. I also helped to take care of my grandfather before he passed. It was one of the hardest things I have done, but it was also one of the most meaningful. It changed how I understood what it means to live a good, meaningful life, and what my duties are as a son, brother, and perhaps more importantly, as a person. A robot can do reminders and check-ins. It is unable to recreate what caregiving does to the caregiver. Across many cultures, including my own, raising children and taking care of your elders are morally serious practices not just because they need help, but because the act of caring forms the character of the caregiver. Handing that off to a machine that simulates the emotional depth of care is not just timesavings. It removes something inherently human from both sides of the relationship.

Physical safety matters here in a way it might not for other products. A 2026 video of a humanoid robot kicking a child at a live demonstration went viral, and it is not hard to see why (Erickson). These are not stationary devices. They move around people, physically assist with mobility, and operate in spaces where the users are already at elevated risk of getting hurt. The IEEE Code of Ethics holds engineers responsible for anticipating foreseeable harm before a product deploys (IEEE). That obligation is more serious when the people in the room are elderly or very young. If something goes mechanically wrong, they have less capacity to get out of harm’s way.

3. Recommended Response

My recommendation is a design rule that companies probably will not like: care robots should have to remain visibly and behaviorally robotic. They should not be designed to look or sound human, and they should not use language that suggests real emotional awareness or affection. A robot can say, “It is time for your medication,” or “Would you like to play a memory game?” It should not say, “I missed you,” “I love spending time with you,” or “You are my friend.” Vallor’s framework stresses honesty and transparency in technology, and a robot designed to create the impression of genuine connection fails that standard.

This is not simply a cosmetic restriction. It is the part of the design that responds to the deception problem. A clearly robotic design will not prevent every possible confusion, especially for dementia patients. But it does change what the company is doing. There is a moral difference between a company trying to reduce confusion and one intentionally designing for it. This also applies to children. Even if a child knows the robot is not alive, continued interaction with a machine that acts like a friend may still shape how they understand what human interaction is supposed to be like.

This rule would not remove the product's useful parts. A robot does not need a human face to remind someone to take medication. It does not need simulated affection to detect a fall, guide a memory game, or alert a caregiver. The human-like features are not necessary for most of the useful benefits. They are useful because they make people more attached to the product. That is exactly why they need ethical limits.

Care robots also should not count toward required human staffing ratios in care facilities. The moment a robot can replace a human caregiver on paper, institutions under financial pressure will have a strong incentive to do exactly that. The robot’s role should be classified as support infrastructure, closer to a medication dispenser or monitoring system than to a nurse. Staffing requirements should remain based on human caregivers.

4. Objection and Reply

The strongest objection is that restricting human likeness may take away the very thing that helps vulnerable people. If a dementia patient is comforted by a robot saying “I missed you,” and that comfort is real to them, then forcing the robot to reveal its artificial nature may seem cruel. Loneliness in old age is incredibly serious. Dementia can be isolating in a way that is hard to understand from the outside. From this perspective, my argument may look like it prioritizes a philosophical standard over real human comfort.

That objection is important, but I think it gives companies too much freedom. The fact that a deception comforts someone does not automatically make it acceptable. Dementia care is very complicated, and caregivers may sometimes choose not to correct every false belief. But that is different from designing a commercial product whose purpose is to create and deepen the confusion by simulating a relationship. The issue is not that the user feels comfort. The issue is that the comfort is produced by taking advantage of a gap in the user’s ability to understand the relationship. A dementia patient who cannot understand their attachment to a robot is not a reason to exploit that attachment. It is a reason to be more careful about creating it. Care robots can still be useful and enjoyable. They just should not be engineered to simulate a relationship that does not actually exist.

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